Lomustine + bevacizumab 7.5 mg/kg

NCCP 00804 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking the treatment

Take lomustine only once on the authorised Day 1. The pharmacist must confirm the total dose, capsule strengths, how to take it and anti-sickness timing. Do not open or crush capsules or take another dose to replace a missed or vomited one without advice. Bevacizumab is given into a vein. The usual first infusion lasts 90 minutes; later infusions may take 60 and then 30 minutes if tolerated. Checks and observation add time. Report flushing, rash, breathing difficulty, dizziness or other new symptoms immediately.

STEP 2

Missed, vomited or held medicine

If a capsule dose is missed or vomited, call the oncology team or pharmacist before taking more. Do not double up or use leftover capsules. An infusion or capsule held for toxicity needs a new team decision.

STEP 3

Medicine and supplement review

Bring all prescription, over-the-counter and herbal medicines, vitamins and supplements to review. Include steroids, seizure medicines, anticoagulants and any recent antibiotics or antifungals. Tell the team before starting or stopping a medicine.

STEP 4

Surgery, procedures and wounds

Tell all treating teams that you receive bevacizumab. Major surgery needs an appropriate treatment interval and a fully healed wound before restarting; minor and dental procedures also need individual advice. Do not decide timing yourself or rely on the wallet stamp.

STEP 5

Capsule supply and handling

Lomustine is supplied as an unlicensed medicine in this NCCP protocol. Confirm pharmacy supply in advance. Keep the labelled pack away from children and do not open or crush capsules. Return unused chemotherapy to the pharmacy; contact staff if swallowing or handling is difficult.

STEP 6

Pregnancy, breastfeeding and fertility

Discuss fertility preservation and contraception before starting. Treatment can harm a pregnancy. Do not breastfeed during treatment. Ask for the written medicine-specific precautions and the required periods after stopping; these may differ between products.

STEP 7

Appointments and transport

Before starting: Blood count, kidney/liver tests, glucose, urine protein, blood pressure and cardiac history/examination; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment. During treatment: Regular blood count, kidney/liver tests and urine protein; blood pressure before each bevacizumab dose and after treatment. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment. MRI and neurological review follow your individual treatment plan. Echocardiography is considered only when indicated, including previous anthracycline treatment or chest-wall radiotherapy. If taking warfarin, INR is checked weekly until stable and before subsequent cycles.

STEP 8

Help with the daily plan

Agree who can help with transport, prescriptions, phone calls and recording symptoms if memory, vision, weakness or fatigue makes them difficult. Your consent and preferences guide what is shared. Keep the printed schedule and actual urgent contacts together.

STEP 9

Off-label treatment and infusion decisions

Bevacizumab for glioblastoma is outside its licensed indication in Ireland. The team should explain the reason for using it, expected benefit, uncertainty and alternatives as part of consent. Any rapid infusion outside the standard schedule has a separate local consent and governance process.

Sources: NCCP 00804 v2 · current Irish regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.